

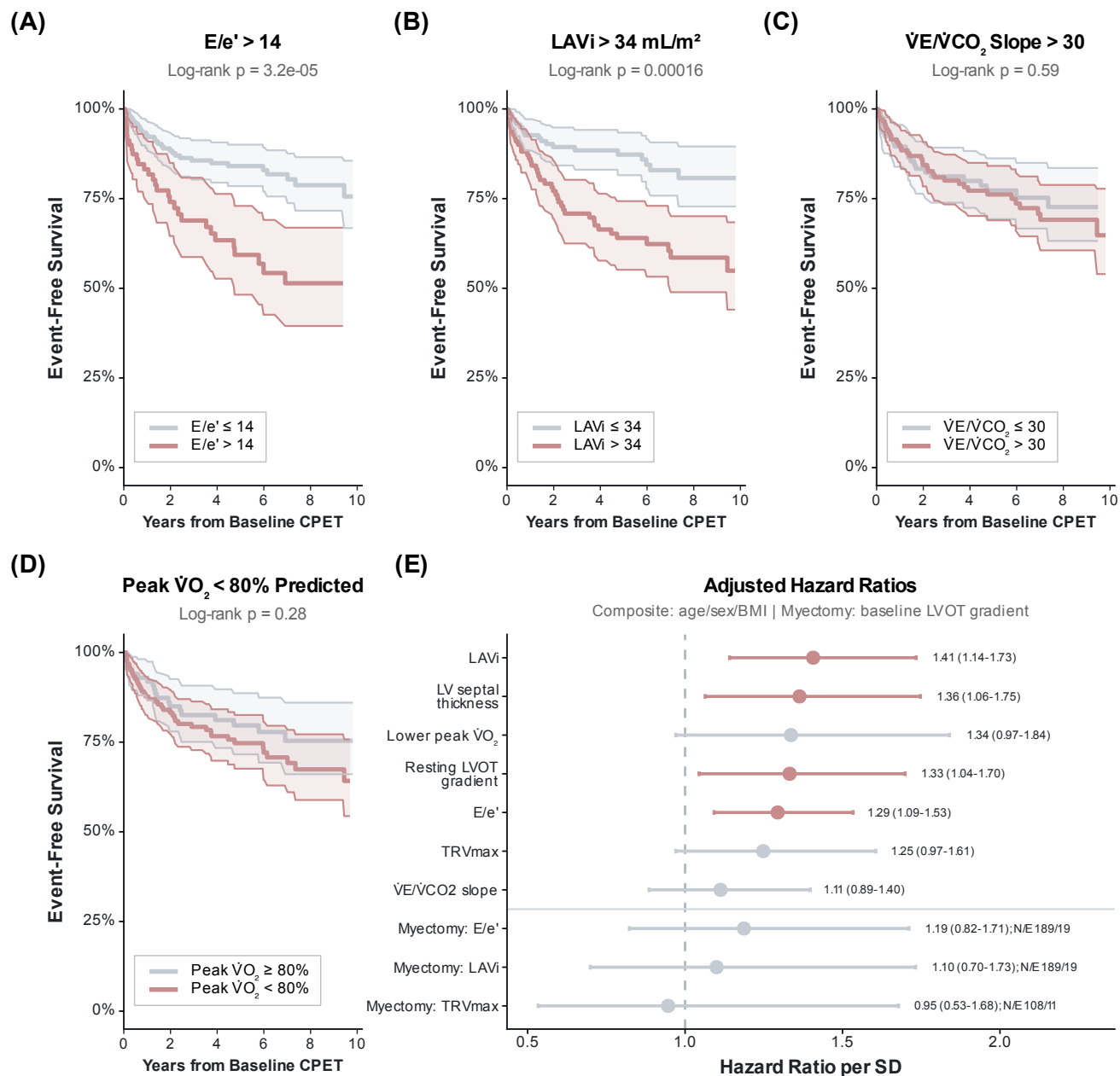


Figure 5. Clinical Outcomes Across LV Diastolic Parameters. The primary endpoint was a dated acute heart failure event, heart transplantation, or death; primary Cox models adjusted for age, sex, and BMI and report hazard ratios per standard deviation; peak $\dot{V}O_2$ was inverse-coded so its hazard ratio is per SD lower fitness. Among 259 patients, 62 first composite events comprised 57 acute heart-failure events, 0 heart transplantation, 5 all-cause deaths, and 0 same-day multiple-component event; 33 surgical myectomies occurred and were treated as a separate secondary outcome. **(A)** Kaplan-Meier event-free survival stratified by $E/e' \leq 14$ versus > 14 . **(B)** Kaplan-Meier event-free survival stratified by $LAVi \leq 34 \text{ mL/m}^2$ versus $> 34 \text{ mL/m}^2$. **(C)** Kaplan-Meier event-free survival stratified by $\dot{V}E/\dot{V}CO_2$ slope ≤ 30 versus > 30 . **(D)** Kaplan-Meier event-free survival stratified by peak $\dot{V}O_2 < 80\%$ versus $\geq 80\%$ FRIEND 2.0 predicted. **(E)** Adjusted hazard ratios (95% CI) for the listed parameters; filled circles indicate $P < 0.05$. Forest-model N/events were E/e' 259/62, $LAVi$ 259/62, TRVmax 144/36, resting LVOT gradient 189/44, LV septal thickness 259/62, peak $\dot{V}O_2$ 259/62, and $\dot{V}E/\dot{V}CO_2$ slope 259/62. The time-constant $\dot{V}E/\dot{V}CO_2$ hazard ratio violated proportional hazards and is interpreted through a supplemental log-time-varying coefficient analysis. The final rows are LVOT-gradient-adjusted myectomy models; generic ablation was omitted because procedure type could not be validated.